DPP	DEPARTMENTAL POLICY AND PROCEDURE		
Title	GAVAGE FEEDING (INTERMITTENT/CONTINUOUS)		
Department	Neonatology Department		
Applies to	All NICU Medical and Nursing Staff		
Policy Code and Version Number	DPP-NICU-018 (v1)		
Last Version Approval Date	N/A	Number of Pages	5

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APPROVAL DATE	EFFECTIVE DATE	TYPE OF REVISION	REVIEW DATE
19 / 3 / 2024	7 / 4 / 2024	TRIENNIAL	18 / 3 / 2027

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1.0 PURPOSE

1.1. To describe the insertion of oral enteral tubes to feed the newborn who cannot safely tolerate oral feedings or who has a contraindication to oral feedings.

2.0 DEFINITION/ABBREVIATION

2.1. **Gavage feeding**: is a procedure in which a tube passed through the nose or mouth (NGT), passed the pharynx, down the esophagus into the stomach used to feed a newborn with weak sucking, uncoordinated sucking and swallowing, respiratory distress, tachypnea, or repeated apneic spells.

3.0 RESPONSIBILITY

2.1. Staff nurse is responsible for safe insertion and removal of gastric gavage, checking of ongoing patency of gastric Gavage and administration of prescribed feeds and medication as prescribed by the physician.

4.0 POLICY

4.1. Gavage feeding in the NICU is where a small, narrow tube is inserted through the infant's nostril and is placed down the throat, esophagus, and into the baby's stomach. This allow for the baby to be fed if it cannot get the nutrition it needs from breast milk or formula alone. The nurse administers intermittent gavage feeding to the infant.

4.2. Feeding tube must be in correct position and properly fixed in the newborn's stomach before commencing gavage feeding. Administering a feeding through a misplaced tube can cause formula to enter the patient's lungs.

4.3. If the newborn gags, chokes, regurgitates, vomits or become cyanotic, the rate of flow of formula is reduced, and the feeding may be stopped.

4.4. Gavage feeding should be given by gravity and should not be pushed by force.

4.5. After giving feeding, the tube should be rinsed with water and close or clamp after rinsing.

5.0 PROCEDURE (S)

5.1. Confirm the physician's order for the feeding, formula/breast milk, volume, frequency of feeding and duration of the feeding.

5.2. Perform hand hygiene.

5.3. Assemble equipment at bedside.

5.4. Verify the identification of the newborn and place supine.

5.5. Remove the feeding tube from the package and measure the distance from the nares to earlobe, and then from the earlobe to halfway between the xiphoid process and the umbilicus.

5.6. Gently and slowly insert the feeding tube through the mouth into the throat. Observe for vagal stimulation (bradycardia, apnea).

5.7. Test for tube placement by aspirating stomach contents with a 3 or 10 ml syringe, or by listening to the stomach with the stethoscope while rapidly injecting 2 – 3 ml of air.

5.8. Remove the tube immediately if the newborn becomes cyanotic, coughing and Fighting the tube or develops symptoms respiratory distress, or confirmation of the tube in the stomach cannot be done.

5.9. Place the duoderm on the newborn's checks to protect the skin from the tape. Tape the tube in position and note the length at the lip.

5.10. **Intermittent Gavage Feedings**:

5.10.1. Prior to feedings, check for gastric aspirate to evaluate digestion as well as to verify the tube remains in the stomach.

5.10.2. The feeding may be placed on a syringe pump over 20 – 30 minutes, or allowed to flow slowly by gravity by elevating the syringe 6 – 8 inches above the level of the newborn.

5.10.3. If the newborn develops signs of distress during the infusion, stop the feeding immediately.

5.10.4. Determine tube position and notify the physician if the distress is not relieved by stopping the feeding.

5.10.5. After giving the feeding, the tube should be rinsed with water (amount of water used to flush the tube is determined by the length of the tube and the age of the baby) and close or clamp after rinsing.

5.10.6. The newborn should be placed in a position of comfort for the feeding so that he/she can be undisturbed after the feeding. Offer non-nutritive sucking if awake.

5.11. **Continuous Gavage Feedings**:

5.11.1. Prime the tubing with formula/breast milk.

5.11.2. Notify the physician

5.11.2.1. If the residual is greater than an hourly volume of feeding

5.11.2.2. If it appears abnormal (coffee ground, bile)

5.11.2.3. If the abdominal girth > 1.5 cm

5.11.2.4. If the bowel sounds are abnormal

5.11.3. Position the newborn in a position of comfort and connect the infusion pump to the feeding tube. Offer non- nutritive sucking if awake.

5.11.4. Program the pump to deliver the ordered rate and volume of feeding.

5.11.5. **Observe for any feeding tolerance and notify the physician:**

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5.11.5.1. Apnea/ bradycardia/desaturations

5.11.5.2. Vomiting

5.11.5.3. Abdominal distention

5.11.5.4. Increasing residuals

5.12. Documentation:

5.12.1. Newborn's tolerance of insertion of the oral gastric tube and the sized of the tube placed.

5.12.2. Date and time the tube was placed, and place a sticker on the tube to confirm date it will be changed (Silastic tubes can be left in place for 21 days).

5.12.3. Intake, type of formula or breast milk and time of feeding on the flow sheet.

5.12.4. Amount of residuals (both discarded and referred).

5.12.5. Abdominal girth every 4 hours or with intermittent feedings.

5.12.6. Quality of bowel sounds and presence/appearance of stool.

5.12.7. Newborn's tolerance of the feedings, including amount of emesis or signs of feeding intolerance.

5.12.8. Adverse symptoms reported to the physician, and the name of the doctor informed

5.12.9. Parent education given and their understanding of the information.

6.0 EQUIPMENTS

6.1. Graduated containers
6.2. Formula/Breast milk
6.3. 60-ml catheter-tipped syringe
6.4. Feeding tube 6, 8 or 10 depends upon the size of the baby
6.5. For continuous tube feeding: Tube feeding bag/syringe and tubing and feeding pump
6.6. Stethoscope
6.7. Gloves
6.8. Tape for securing tape

7.0 ATTACHMENT/FORMS

7.1. Daily Intake and Output Record (GDOH-NUR-DIOR-189)
7.2. Nurses Progress Notes (GDOH-NUR-NPN-203)
7.3. Nurse Observation Form (Form No. 43)
7.4. Interdisciplinary Patient/ Family education Form (GDOH-COR-IPFERF-349)

8.0 REFERENCES

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8.1. Cloherty JP, Eichenewald EC and Start AR (2008). Manual of Neonatal care, 6th Ed. Philadelphia: Wolters Kluwer Lippincott Williams & Wilkins.

8.2. MacDonald MG and Ramasethu J (2007). Atlas of procedure in Neonatology,4th Ed. Philadelphia: Wolters Kluwer Lippincott Williams & Wilkins.

8.3. Gardner SL.Carter BS,Enzman-Hines M and Hermandez JA(2011). Merenstein & Gardners Handbook of Neonatal intensive care ,7th Ed.St.Louis:Mosby Elsevier

9.0 REVISION CLAUSE

9.1. N/A

KINGDOM OF SAUDI ARABIA

وزارة الصحة
Ministry of Health

Hospital: _____ :مستشفى

Region: _____ :المنطقة/المحافظة

Dept./Unit: _____ :القسم/الوحدة

MRN: _____ :رقم الملف الطبي

Name: _____ :الاسم

Nationality: _____ :الجنسية

Age: _____ ☐ سنة Years ☐ شهر Months ☐ يوم Days :العمر

Date of Birth: ___/___/14___ H ___/___/20___ :تاريخ الميلاد

Gender: ☐ Male ☐ Female :الجنس

DAILY INTAKE AND OUTPUT RECORD

Date: **Height:** **Weight:**

INTAKE								OUTPUT								
Time	Intravenous Fluid 1 Type	Intravenous Fluid 1 Rate	Intravenous Fluid 2 Type	Intravenous Fluid 2 Rate	Blood	Oral/ Tubes	Total In	Time	Urine	Emesis / NGT	DRAIN 1	DRAIN 2	BM Diarrhea	Blood loss	Total Out	RN Initial/ Job number
0800								0800								
0900								0900								
1000								1000								
1100								1100								
1200								1200								
1300								1300								
1400								1400								
1500								1500								
Total 8HRS								Total 8HRS								
1600								1600								
1700								1700								
1800								1800								
1900								1900								
2000								2000								
2100								2100								
2200								2200								
2300								2300								
Total 8HRS								Total 8HRS								
2400								2400								
0100								0100								
0200								0200								
0300								0300								
0400								0400								
0500								0500								
0600								0600								
0700								0700								
Total 8HRS								Total 8HRS								
Total 24HRS								Total 24HRS								

TOTAL INTAKE 24 HOURS _____

TOTAL OUTPUT 24 HOURS _____ **24 Hours (+ / -) Balance:** _____

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KINGDOM OF SAUDI ARABIA

وزارة الصحة
Ministry of Health

Hospital: _____ مستشفى:

Region: _____ المنطقة/المحافظة:

Dept./Unit: _____ القسم/الوحدة:

MRN: | | | | | | | | | | رقم الملف الطبي:

Name: _____ الاسم:

Nationality: _____ الجنسية:

Age: _____ ☐ سنة Years ☐ شهر Months ☐ يوم Days العمر:

Date of Birth: ____ / ____ / 14____ H ____ / ____ / 20____ تاريخ الميلاد:

Gender: ☐ Male ☐ Female الجنس:

NURSES PROGRESS NOTES FORM

DATE &TIME	NURSES NOTES (D-data A-action R-response)	REGISTERED NURSE INITIAL WITH ID NUMBER

Note: Write the time in each entry & affix your initial at the end of each paragraph. Document your complete Name, Initial, Job number, Date & Time at the closure of your documentation. Draw a line across empty spaces.

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Ministry of Health
General Directorate of Health Affairs
Al-Baha

وزارة الصحة
المديرية العامة للشئون الصحية بمنطقة الباحة

HOSPITAL :

Health Care No .: ☐☐☐☐☐☐☐☐☐☐☐☐

Name : .....

Age : ☐☐ Yrs./Month Sex : ☐M ☐F

Nationality : .....

Consultant In-Charge : .....

Dept : ..... Unit : .....

NURSES OBSERVATION FORM نمـوذج مـلاحظـات التمـريضـ

Nurse / Date	Time	Temp.	Puls Rate	Resp Rate	B.P.	Pupils		Level of Consciousness Muscle Tone &Reflex	Cough Reflex	Response to Stimuli		Perception Pulses			Skin Color	Rema--rks	Sign.
						R	L			Verbal	Painful	ORAL	Audio Aural	visual			

SPECIFIC NURSING CARE COMMENTS :

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Dept./Unit: _____ القسم/الوحدة:

MRN: | | | | | | | | | | رقم الملف الطبي:

Name: _____ الاسم:

Nationality: _____ الجنسية:

Age: _____ ☐ سنة Years ☐ شهر Months ☐ يوم Days العمر:

Date of Birth: ____ / ____ / 14____ H ____ / ____ / 20____ تاريخ الميلاد:

Gender: ☐ Male ☐ Female الجنس:

INTERDISCIPLINARY PATIENT / FAMILY EDUCATION RECORD FORM

Part 1: Education Needs Assessment: (Write Number codes as applicable) ☐ **Out Patient** ☐ **In-Patient**

1- Personal Hygiene
2- Pain Management
3- Activity / Exercise
4- Disease Process
5- Safe & Effective Use of Medical Equipment
6- Diagnostic Test / Procedure
7- Health Maintenance
8- Social Services
9- Safe & Effective Use of Medication
10- Discharge Instruction
11- Nutrition
12- Risk
13- Safety
14- Consent
15- Others(Specify)_____

Part 2: CODES:

Learning Barriers:

1- No Learning Barriers
2- Impaired Hearing
3- Speech Barriers
4- Emotional Barriers
5- Language Barrier
6- Educational Level
7- Desire/Motivation to Learn
8- Impaired Thought Process
9- Responsibilities at Home
10- Cultural differences
11- Religious Practice
12- Impaired Vision
13- Financial Difficulties
14- Others: (Specify) _____

Response to Teaching:

0. Not Receptive to teaching
1.Verbalizes Understanding
2.Demonstrated Ability / Understanding
3.Needs Follow-up

Person Taught:

PT: Patient
Sn: Son
SO: Significant Others (Specify)
S: Spouse
F: Father
M: Mother
D: Daughter

Teaching Tools:

A: Audio
O: Oral
V: Video
W: Written instructions
P: Printed Materials Provided
D: Demonstration

Part 3: Education Process

Date	Time	Education Needs	Information Taught	Learning Barriers	Person Taught	Teaching Tools	Response to Teaching	Evaluation /Comments	Informant		
									Name	Dep.	Signature/ ID number

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رقم الملف الطبي: MRN:	الاسم: Name:

نموذج تثقيف المريض/العائلة

الجزء الاول: تقييم احتياجات التثقيف (يرجي كتابة الرموز كما هو مطبق) ☐ التنويم ☐ العيادات الخارجية

١- العناية الشخصية	٤- المرض	٧- المحافظة على الصحة	١٠- تعليمات الخروج	١٣- الأمان
٢- التعامل مع الألم	٥- الاستخدام الآمن والفعال للأجهزة الطبية	٨- الخدمات الاجتماعية	١١- التغذية	١٤- إقرار الموافقة
٣ - النشاط/ التمارين الرياضيه	٦- التحاليل / الفحوصات	٩- الاستخدام الآمن والفعال للدواء	١٢- المخاطر	١٥- أخرى (حدد)

الجزء الثاني : (الرموز)

عوائق التثقيف

١- لا توجد عوائق	٥- صعوبات لغة	٩- مسؤوليات منزلية	١٣- ضعف بصري
٢- ضعف سمعي	٦- المستوى التعليمي	١٠- الاختلافات الثقافية	١٤- أخرى (حدد)
٣- صعوبات نطق	٧- الرغبة / الحافز للتعلم	١١- الصعوبات المالية	
٤- صعوبات نفسية	٨- عملية تعلم ضعيفة	١٢- الممارسات الدينية	

الاستجابة للتثقيف	الشخص الذي تم تثقيفه		أدوات التثقيف	
٠- لا يستجيب للتثقيف	PT- المريض	D- الابنة	A- مسموع	O- شفهي
١- الفهم الشفهي	S- الزوجة / الزوج	SN- الابن	V- مرئي	W- تعليمات خطية
٢- القدرة على التطبيق	F- الوالد	- أخرى SO (حدد)	P- مواد مطبوعة	
٣- يحتاج للمتابعة	M- الوالدة		التوضيح العملي D	

الجزء الثالث : آلية التثقيف

التاريخ	التوقيت	احتياجات التثقيف	المعلومات المعطاة	عوائق التثقيف	الشخص المثقف	أدوات التثقيف	الاستجابة للتثقيف	الملاحظات/ التقييم	المثقف		
									الاسم	القسم	التوقيع

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